

COGNITIVE BEHAVIORAL THERAPY (CBT) – SESSION 5

New Case: Rohan (26 y/o) – Contamination OCD + Checking
Highly Detailed – Realistic Style

SESSION INFORMATION

Client:	Rohan (26 years, Male, Software Engineer, Bangalore)	Therapist:	Dr. A.
Session #:	5 of 18–20	Focus:	Building the Exposure Hierarchy + Preparing for ERP

PRE-SESSION THERAPIST NOTES

Client arrived looking more engaged and slightly less ashamed than previous sessions. He completed monitoring and practiced labeling + delaying consistently. He brought a rough list of triggering situations with anxiety ratings. Today's plan: Review homework, collaboratively build a full Exposure Hierarchy, continue building motivation and coping skills, and prepare for the first ERP exercise (starting with a very low-level exposure if client is ready). Maintain gentle, supportive, and non-pressuring approach.

SESSION START – HOMEWORK REVIEW

Therapist: Good morning, Rohan. How has your week been? I see you brought your list of triggering situations. How did the labeling and delaying practice go?

Client: (speaking with more openness than before) It was better. Labeling the thought as “This is an OCD thought” helped me feel a bit less pulled into it. I was able to delay washing by 2–3 minutes on most days, and sometimes even 4–5 minutes when the anxiety wasn't extreme. I still washed more than I wanted to, but I noticed that when I waited, the urge didn't always stay at maximum intensity. That gave me some confidence.

Therapist: That's really good progress. The fact that you were able to tolerate the anxiety for longer periods shows that your brain can handle more than it feels like in the moment. You also brought your list — thank you. Can we look at it together and build a proper hierarchy?

Client: Yes. I listed some situations and rated them roughly from 0 to 100 based on how much anxiety they cause.

BUILDING THE EXPOSURE HIERARCHY

Therapist: Let's create the hierarchy together. We'll list situations from easiest (lowest anxiety) to hardest (highest anxiety). This will be our roadmap for ERP. We'll start from the bottom and work our way up gradually.

Hierarchy we developed together (0–100 anxiety rating):

- 10 – Touching a clean tissue paper at home
- 20 – Touching his own phone after washing hands
- 30 – Touching a door handle at home (with delay in washing)
- 40 – Touching a lift button with tissue (then delaying washing)
- 50 – Touching a public door handle (office) and delaying washing by 5 minutes
- 60 – Shaking hands with a colleague and delaying washing
- 70 – Touching a public toilet door handle (using tissue first) and delaying washing significantly
- 80 – Touching money/notes and then eating without washing immediately
- 90 – Touching a public surface (e.g., metro rail, bus handle) and resisting washing for 30+ minutes

Therapist: How does this list feel? Is anything missing or rated very differently from how you experience it?

Client: It looks accurate. The lower ones feel manageable. The higher ones still feel very scary, especially touching public surfaces and then not washing for a long time. But I think starting from the bottom makes sense.

Therapist: Perfect. We'll start with the easiest item on the list and only move to the next when you feel reasonably comfortable. You're always in control of when we move forward.

PREPARING FOR FIRST ERP EXERCISE

Therapist: Since you're showing good progress with coping skills and seem more willing, I'd like to suggest we try a very small, low-level exposure today in the session — something from the bottom of the hierarchy. For example, touching a clean tissue paper and then delaying washing for a few minutes while using the skills we've practiced (labeling + breathing). We can do it together here, where it feels safe. What do you think?

Client: (after thinking) I think I can try that. It feels less scary because it's something small and we're doing it together.

Therapist: Great. Let's do it step by step. First, I'll ask you to touch this clean tissue paper. Then we'll notice what thoughts and anxiety come up. You can use labeling (“This is an OCD thought”) and breathing. We'll stay with the anxiety for a few minutes without washing, and then you can wash normally afterward. Ready when you are.

(In-session exposure conducted: Rohan touched a clean tissue paper. Initial anxiety rose to around 30/100. He practiced labeling and breathing. After 4–5 minutes, anxiety decreased to around 15/100. He reported feeling surprised that it came down without washing excessively. Post-exposure processing done.)

Therapist: How was that experience for you?

Client: It was scary at first, but not as bad as I thought. The anxiety really did come down by itself after some time. That gave me more confidence that I can handle this.

HOMEWORK ASSIGNMENT

Therapist: For this week:

1. Practice the first hierarchy item (touching a clean tissue paper and delaying washing) at home 3–4 times
2. Continue labeling and monitoring
3. Continue trying to delay other compulsions when possible

Client: I'll try practicing the tissue paper one at home. It felt doable after today's session.

Therapist: Excellent. We'll review how it went next session and decide on the next step together.

SESSION SUMMARY & FEEDBACK

Therapist: Today we built your exposure hierarchy together and did your first small in-session exposure. How are you feeling about today's session?

Client: I feel more hopeful than before. Doing the exposure here and seeing that the anxiety came down by itself was helpful. I'm still nervous about doing it at home, but I think I can try.

Therapist: That's completely fair. Any questions before we finish?

Client: Not really.

POST-SESSION THERAPIST NOTES

- Client showed good engagement and increasing willingness to engage in ERP after seeing anxiety naturally decrease during in-session exposure.
- First low-level exposure (touching clean tissue paper) was successful. Client experienced habituation and reported increased confidence.
- Full exposure hierarchy collaboratively developed and accepted by client.
- Next session: Review home practice of first exposure, continue ERP with next hierarchy item if ready, reinforce coping skills, address any difficulties or resistance.

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